



OFFICIAL

NMHS MHS PROCEDURE

Mental Health Consumers Who Are Missing or Absent without Leave

Scope (Audience)	All NMHS Mental Health Service (MHS) Staff
Scope (Area)	All NMHS MHS Inpatient, State Forensic and Community Mental Health Service Areas
Summary	This procedure sets out the mandatory requires for NMHS MHS under the Mental Health Policy Framework to: • reduce the risk of mental health consumers becoming absent without leave (AWOL) or a missing person • ensure the safety of mental health consumers who are missing or AWOL • ensure compliance with the Mental Health Act 2014 (MHA 2014)
Risk Rating	Medium
NSQHS Standards	Clinical Governance (Std 1) Communicating for Safety (Std 6)
Policy Sponsor	Director of Medical Services MHS
Policy Contact	Directors of Inpatient MHS and Director of Nursing MHS
Version	4.0
Effective Date	11 November 2025
Review Date	11 November 2027

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

This procedure provides a structured clinical response for instances where a mental health consumer is missing or suspected to be missing. It aims to support timely risk assessment, ensure appropriate escalation and notification, and promote consumer safety ensuring compliance with the [Mental Health Act 2014](#) and in accordance with the Department of Health's [Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#) and its supporting procedures.

2. Background

North Metropolitan Health Service Mental Health Services (NMHS MHS) manages missing consumers in accordance with:

- Department of Health (DoH) MP 0012/16 [Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#)
 - [DoH Procedures for Involving Police When Mental Health Consumers Are Missing or Absent without Leave](#)
- [Best Practice Guideline: Mental Health Consumers Who are Missing or Absent Without Leave](#)
- [DoH MP 0122/19 Clinical Incident Management Policy 2019](#)
- [DoH MP 0181/24 Safety Planning for Mental Health Consumers Policy](#).
- [DoH MP 0155/21 State-wide Standardised Clinical Documentation for Mental Health Services](#)
- [DoH MP 0063/17 Road Based Transportation for Mental Health Consumers Policy](#)
- [DoH MP 0099/18 Community Mental Health Status Assessments: Role of the Mental Health Clinicians Policy](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#).

All clinical incidents (including where a patient is missing or suspected of being missing) must be documented in the patient's health records and registered as a clinical incident via Datix CIMS in accordance with the [MP 0122/19 Clinical Incident Management Policy 2019](#) and the "Missing Patients Event Flowchart" (**Appendix 1**) These actions must be followed to ensure the notification of appropriate staff. For Determination of SAC rating refer to Datix CIMS Business Rule on Determination of SAC rating (**Appendix 2**).

3. Risk

Failure to follow this procedure may result in a risk to patient well-being, safety and outcomes, as well as the reputation of the organisation and failure to make appropriate notifications.

Non-compliance with this procedure will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☒	Workforce Planning	☒
Medical Equipment	☐	Other	☐

4. Language and Definitions in Reporting to the WA Police Force

WA Police Force Category	Key Elements	Reporting Guidance
Absent	- Whereabouts unknown. - No genuine concern for safety. - No suspicion of criminality. - Not where expected.	Do not report to police.
Absent Without Leave (AWOL)	- A patient subject to the Mental Health Act 2014, CLIMI Act 2023, or Prisons Act 1981 who is absent without lawful permission. - Includes failure to return from leave or absconding while under an order. - May or may not meet police criteria for "Missing".	- Notify WA Police if patient is at risk or as per legal duty (e.g. ARO issued). - Notify next of kin, MIARB (if required), and record in clinical documentation. - Complete AWOL/Missing Patient forms per policy. - Inform MHERL
Lost	- Person is unaccounted for and disoriented/incapacitated. - Last known location identified. - Distress or urgency applies.	- Emergency: call 000 and ask for police. - Non-emergency: call police on 131 444.
Missing	- Whereabouts unknown. - Genuine and serious concern for the person's safety/welfare. - May involve suspicion of criminality (e.g. abduction).	- Emergency: call 000 and ask for police. - Non-emergency: call police on 131 444. - Inform MHERL

5. Procedures – Inpatient Mental Health Services

5.1 Inpatient setting

- Inpatients' whereabouts must be actively monitored and documented.
- All inpatients must have a current risk assessment recorded in their clinical documentation, which includes consideration of their risk of absconding.
- Where an inpatient is identified as being at risk of absconding, a risk management plan must be developed, documented in the health record, and communicated to relevant staff using iSoBAR handover principles.
- If an inpatient is found to be absent or suspected to be missing, staff must undertake and clearly document all "*diligent efforts*" to locate the patient prior to initiating escalation. This includes:
 - A thorough search of the ward and surrounding internal areas.
 - Requesting hospital security to assist with a coordinated search of the hospital grounds.

- Attempting direct contact with the consumer.
- If applicable contacting Aboriginal Mental Health Liaison Services via email NMHS.MHAboriginalHLP@health.wa.gov.au for assistance.
- Checking known locations or areas of interest.
- Consulting with family or personal supports where appropriate.

These actions must be documented in the clinical record as part of the initial response and reviewed by the shift coordinator and relevant medical officer.

5.1.1 Involuntary Patients

- Any patient detained under the *MHA 2014* must be considered at risk if they abscond or are suspected to be missing from an inpatient unit. The admitting Consultant Psychiatrist or the on-call Consultant Psychiatrist (after hours) must be notified within (one)1 hour of the patient being identified as missing.
- The Consultant Psychiatrist must assess whether the criteria for Absent Without Leave (AWOL) under s. 97 of the *MHA 2014* are met. If so, an [Apprehension and Return Order \(Form 7D\)](#) must be completed by the treating or duty medical officer.
- Where practicable, photographic identification of the patient should be provided to services assisting with the search or return of the patient, such as the WA Police Force or relevant community services, in accordance with privacy and clinical risk considerations.
- On return to the hospital, formal patient identification must occur in accordance with the [NMHS MH Identification of Patient Procedure](#).
- In line with [MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#) carers, family members, or nominated support persons should be notified of the consumer's absence or return where clinically appropriate and safe to do so. Staff must consider the patient's care preferences, safety, risk, and cultural or trauma-informed factors when determining who to notify.

5.1.2 Initial Interventions when a patient is suspected missing

- Staff must be aware of each patient's absconding risk and clinical observation level, as outlined in the [NMHS MH Clinical Observation Levels Procedure](#) and [MH Clinical Risk Assessment and Management Policy](#).
- If a patient is suspected to be missing:
 - Follow the "AWOL / Missing Decision Flowchart" (**Appendix 1**).
 - The staff member identifying the absence must immediately inform the Shift Coordinator, regardless of discipline.

5.1.3 Staff Safety and Observation Responsibilities

- Staff are not expected to follow or restrain absconding patients. They should observe and note the direction the patient is heading, their appearance, any distinguishing features of the patient and immediately inform the ward/unit.

5.1.4 Escalation of Notification

- The Shift Coordinator will escalate as follows:
 - Notify the Clinical Nurse Specialist/Manager.
 - Notify Consultant Psychiatrist (CP) or CP on-call.
 - Notify the Coordinator of Nursing (after hours), who may escalate to the Executive on Call (EOC) if required.

5.1.5 Search Procedures

- Security must be requested to assist with a search of the hospital grounds.

- Initial search must include the ward/unit, toilet, courtyards, public areas and immediate hospital grounds.
- Broaden search based on the individual's assessed risk.

5.1.6 Clinical decision making

The treating team will assess:

- The patient's current mental state and known risks.
- Their history of absconding or harm to self/others.
- Legal status under the *MHA 2014*.
- Potential locations, including checking the patient's residence.
- Whether WA Police assistance is urgently required.
- Need for welfare check.

5.1.7 Welfare checks

The treating team will assess:

- The patient's current mental state and known risks.
- Their history of absconding or harm to self/others.
- Legal status under the *MHA 2014*.
- Potential locations, including checking the patient's residence.
- Whether WA Police assistance is urgently required.

5.1.8 Patients, not considered to be "At Risk"

- Make a reasonable attempt to contact the patient and confirm their whereabouts.
- Notify the treating CP and review the patient's legal status under the *MHA 2014*.
- A decision may be made to discharge the patient if they are not deemed at risk.
- Inform the Next-of-kin/PSP of the absence and discharge decision, if applicable.
- Document all actions and communications clearly.

5.1.9 Patients, considered to be "At Risk"

- Make every effort to locate the patient, including contacting their phone, support people, or known contacts.
- Notify the CP as soon as possible.
- Notify the PSP and request information on possible whereabouts.
- Notify WA Police as soon as practicable, including:
 - Patient description, clothing, distinguishing features.
 - Clinical concerns regarding risk.
 - Provide a photograph where available.
- Document the report on the appropriate form (e.g., Missing Patient Form).
 - Must be signed by or referenced to the duty/team medical officer.
 - Obtain and record WA Police Job Number.
- Notify the relevant community mental health service or MHERL/ATT as appropriate.
- If the patient is found, notify community staff or ATT and arrange for return to hospital.
- Notify CP, PSP, and WA Police of the patient's return.

5.1.10 Involuntary patients considered "At Risk" or whose location is unknown

- Immediately notify WA Police, providing:
 - Patient's description, photo, clothing, risk details.
- Notify ATT or community teams depending on business hours.
- Complete Missing Patient or AWOL form (signed or authorised by medical officer).
- Notify the carer/PSP in line with s. 97 of the *MHA 2014*.
- A medical practitioner will complete an Apprehension and Return Order (ARO, [Form 7D](#)).
 - Remains valid for 14 days.
- An ARO can also be issued for patients under a **Form 1A** who abscond from unauthorised facilities (e.g., ED).

5.1.11 Location, transport and safe return of patient

- Refer to [MP 0063/17 –Road-Based Transport](#).
- Complete [Form 4A](#) and transport risk assessment as required.
- Request WA Police or ambulance support only when clinically necessary due to risk.
- On return, the patient must be:
 - Formally identified (photo or two clinicians who know the patient).
 - Clinically assessed (mental and physical state) by the Team/Duty Medical Officer.
- Document the identification, assessment, and care plan updates in the health record.
- Notify next-of-kin/PSP and all relevant parties of the patient's return.

5.1.12 Notification procedure when a patient is AWOL

- Follow the “AWOL / Missing Decision Flowchart” (**Appendix 1**).
- WA Police are only to be involved when risk remains after clinical interventions/strategies applied.
- The treating doctor must report AWOL to Coordinator of Nursing and Head of Service with escalation to Co-Director (or Coordinator of Nursing after hours).
- Notify the Head of Service/Co-Director when the patient is placed AWOL and when they are located.
- Refer to:
 - [Chief Psychiatrist's Notifiable Incident Reporting Requirements](#).
 - [NMHS Clinical Incident Management Policy and Procedure](#).

6. Procedures – State Forensic Mental Health Services (SFMHS)

6.1 Frankland Centre & Dryandra Ward

The Frankland Centre is a secure forensic mental health inpatient facility responsible for the care and custody of patients who are subject to legal orders under the [Criminal Law \(Mental Impairment\) Act 2023](#) (CLMI Act), [Mental Health Act WA 2014](#) (MHA 2014), the [Prisons Act 1981](#) (WA), and related legislation. This includes patients on Custody Orders (CO) issued following a judicial determination of unfitness to stand trial or a finding of unsoundness of mind, as well as prisoners receiving treatment under the *MHA 2014* and statutory provisions such as s. 83 of the Prisons Act.

Dryandra Ward is a low secure forensic rehabilitation ward that includes patients on Custody Orders (CO) who have frequent unescorted ground and community access. Some of these patients live in the community on a part time basis while transitioning towards discharge.

Patients under these orders are considered to be in legal custody, and any absence from the Frankland Centre and Dryandra Ward without legal authorisation may constitute absconding or escape from custody, triggering mandatory notification and escalation requirements.

The Mental Impairment Review Tribunal (MIRT) authorises a Leave of Absence order (LOA) for appropriate CO Patients. The CO patient that is unable to comply with the LOA orders may be assessed to have breached that order which will result in an escalation of plans for that patient to be returned to Frankland or Dryandra wards.

6.2 Absconders: Absent without Leave (AWOL)

All patients authorised to be on ground and community access will be clearly documented as per the [Inpatient: Hospital Campus Access and Community Outings](#).

If a patient has failed to return to the ward environment at the stipulated time, then the escalation pathway will be engaged as per the AWOL process.

Routine headcounts must occur at least hourly within the Frankland Centre and during all transitions between areas. If a patient is discovered to be missing, staff must immediately:

- Notify the Senior Nurse in Charge, CNS/CNM.
- Initiate a coordinated search of the ward and surrounding areas, led by nursing and Allied Health staff.
- Request security assistance and maintain communications via two-way radio under the direction of the CNS/CNM or Senior Nurse in Charge.
- If the patient is not located internally, a search of the hospital precinct and nearby grounds will be conducted by a minimum of three staff, including at least one security officer.
- The CNS/CNM or Senior Nurse in Charge will complete and initiate notifications using the 'Absent Without Leave Form', documenting all external agencies and individuals contacted.
- The patient's next-of-kin or identified personal support person (PSP) must be notified, and the contact documented in the clinical record.
- Notify the relevant authorities (as above) including community mental health teams if appropriate during business hours or the Mental Health Emergency Response Line (MHERL) after hours.

6.3 Escapes from Legal Custody - Patients under the CLMI Legislation

6.3.1 Escalation pathway when a CO patient may be in breach of the LOA

- The CNS or CNM After hours will be the first point of contact at the Frankland Centre.
- They will coordinate the notification procedure for the safe return or apprehension of the CO patient.

6.3.2 Business Hours Escalation

The treating medical practitioner, senior clinician in charge, or other suitably qualified health professional of the SFMHS or other HSP may contact the Supervising Officer or other Department of Justice representative by phone and / or email depending on the degree of urgency / severity and formulated level of risk.

Adult Community Corrections CLMI Team:	Youth Justice Service CLMI Team:
Phone: 08 9227 2577	Phone: 08 9227 2500
Email: acc.clmi@justice.wa.gov.au	Email: yjs-clmi@justice.wa.gov.au

6.3.3 After-Hours Escalation (For Urgent Matters)

The treating medical practitioner, senior clinician in charge, or other suitably qualified representative of the SFMHS or other HSP may contact the on-call Adult Community Corrections (ACC) or Youth Justice Service (YJS) manager or team leader for time-critical / urgent / high severity / high risk breach incidents.

Adult Community Corrections CLMI Team: Phone: 0413 677 083 (Manager) 0427 459 636 (Team Leader)	Youth Justice Service CLMI Team: Phone: 0474 164 201
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6.4 Prisoners under MHA

Patients who are prisoners under the *Prisons Act 1981 (WA)* receiving care at the Frankland Centre under the *MHA 2014* are in legal custody. If such patients abscond:

- The CNS/CNM or Senior Nurse in Charge must notify the WA Police Force immediately via 000 to report an “*Escape from Legal Custody*”.
- The Prisons Operations Centre must also be notified directly by phone.

6.5 Communications with WA Police Force

- All communications with the WA Police Force are the responsibility of the CNS/CNM or Senior Nurse in Charge.
- When reporting, refer to the individual as an “*an escape from legal custody*” to align with police procedures.
- Required police information includes:
 - Full name, Date of Birth and address.
 - Physical description and clothing.
 - Legal status and assessed risk to self and others) to be confirmed with the treating or on-call CP).
 - Last known time and place the patient was seen.

6.6 Return of a Patient

Upon return:

- The CNS or Senior Nurse in Charge must notify all relevant parties listed on the ‘*Return from AWOL Form*’.
- A physical and mental health assessment must be completed and documented immediately by the medical and/or nursing staff.
- Risk status and custody arrangements should be reviewed without delay.

6.7 Review of Process

A review of the circumstances surrounding the absconding should occur as soon as possible via the members of the clinical incident review and immediate interventions applied as required. Incident debriefing process should be implemented as per policy.

7. Procedures – Community Mental Health Services

7.1 Community Consumer

When a staff member becomes aware that a consumer is absent from their usual place of residence or fails to attend a scheduled appointment, they must consult with the treating team to assess whether the non-attendance raises concern regarding the consumer’s safety or

wellbeing. This assessment must be documented in the health record and should include consideration of the consumer's mental state, risk history, and known patterns of behaviour.

If it is determined that there is a genuine and serious concern, reasonable efforts must be made to determine the consumer's whereabouts and confirm their safety. These actions must be documented as diligent efforts in the clinical record.

7.2 Voluntary Consumers in the Community

Voluntary patients are not compelled to engage with services under the *MHA 2014*. However, where there is concern for a consumer's welfare due to non-attendance or being uncontactable, staff must undertake reasonable efforts to ascertain their wellbeing. This may include:

- Attempting to phone the consumer on known contact numbers.
- Conducting a home visit if safe and appropriate.
- Contacting next-of-kin, family, or nominated support person.
- Making discreet enquiries with neighbours or other contacts (if clinically indicated and confidentiality considerations permit).
- Notifying or seeking support from the Mental Health Emergency Response Line (MHERL) or Assessment and Treatment Team (ATT) if the risk is considered urgent.

Note: Police must not be contacted unless the absence meets the WA Police criteria for a missing person: i.e. where there is a genuine and serious concern for the person's welfare or a suspicion of criminality. This aligns with s. 3.4 of the Mental Health Consumers Who Are Missing or Absent Without Leave Policy.

7.3 Involuntary Patient in the Community

For consumers subject to a Community Treatment Order (CTO) under the *MHA 2014*, non-attendance at scheduled treatment appointments or being absent from a required location must be escalated to the supervising Consultant Psychiatrist.

The treating team must:

- Review the consumer's clinical risk assessment and CTO conditions.
- Attempt to contact the consumer via phone or visit their usual residence.
- Contact next-of-kin or nominated persons for information about the consumer's whereabouts.
- Refer to MHERL or ATT for assistance if appropriate.
- Document all actions as diligent efforts in the health record.

Where the absence is assessed as a breach of the CTO, the supervising Consultant Psychiatrist may consider revocation of the order under s. 131 *MHA 2014*.

7.3.1 Non-Attendance by Community Consumers

- Consumers receiving care from a NMHS Mental Health Community Services may occasionally be absent from their usual place of residence or fail to attend scheduled appointments or activities. This may occur for various reasons and does not automatically indicate risk or require emergency escalation.
- Regardless of their legal status under the *MHA 2014*, all community mental health consumers must have a current risk assessment documented in their clinical record.

- If a consumer is not present during a scheduled home/community visit or fails to attend a scheduled appointment or activity (see Sections 7.2 and 7.3), the response should be informed by:
 - The consumer's known clinical risks.
 - The nature of the missed contact.
 - Recent changes in presentation or risk behaviours.
- Staff must undertake and document all "diligent efforts" to make contact or determine the consumer's safety. This may include:
 - Calling the consumer or their carers/support persons.
 - Reviewing appointment logs and relevant health information systems.
 - Checking with hospitals, emergency departments, or known providers.
- If a consumer cannot be located and there is a genuine concern for their safety, refer to the [Did Not Attend \(DNA\) Community Appointment Procedure](#), the clinician must assess whether the situation meets the WA Police definition of a missing person. If so, escalation must occur in line with the [Procedures for Involving Police When Mental Health Consumers Are Missing or Absent Without Leave](#)

7.3.2 Voluntary Consumer in the Community

- Notwithstanding a voluntary consumer's right to make independent decisions regarding their engagement in treatment, staff have a duty of care to assess safety and risk when a consumer is absent from their residence or fails to attend a scheduled appointment.
- In such cases, the community mental health clinician must consult with the treating team to:
 - Determine whether the absence is clinically concerning.
 - Review recent presentation and risk factors.
 - Decide on appropriate next steps (e.g. follow-up call, home visit, no further action).
- Importantly, where there is no identified risk to the consumer or others, police must not be contacted. This is consistent with s.3.4 of [MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy](#), which states that non-attendance by a voluntary patient, in the absence of safety concerns, does not meet the threshold for police notification.
- All decision-making and actions taken must be documented in the health record, including consultation with the treating team, attempts to contact the consumer, and rationale for escalation or non-escalation.

7.3.3 Involuntary Consumer in the Community

- A consumer receiving care under a Community Treatment Order (CTO) will have the conditions of that order documented in accordance with s. 115 of the *MHA 2014* this includes required times, locations and modalities of treatment.
- If a consumer fails to attend a scheduled treatment appointment or is present during a planned home/community visit, the Consultant Psychiatrist (CP) responsible for supervising the CTO must be notified without delay
- A missed appointment or non-attendance may constitute breach of the CTO, depending on the specific conditions of the order. The criteria for a breach are outlined in s.126 of the *MHA 2014*.
- All actions and clinical reasoning must be clearly documented in the consumer's health record, including any contact attempts, consultation with the treating team, and decisions regarding the CTO compliance or revocation.

8. Death of a Patient / Consumer

- If it is confirmed that a previously missing patient or consumer is deceased, staff must immediately follow relevant local health service protocols and initiate the appropriate reporting processes. This includes:
- Complying with the [NMHS Mental Health Death of a Patient Procedure](#).
- Completing required notifications in accordance with the [MP 0122/19 Department of Health Clinical Incident Management Policy](#).
- Submitting a Datix CIMS report and any other required documentation under notifiable incident guidelines.
- Notifying senior staff including the Consultant Psychiatrist, Coordinator of Nursing, and relevant Executive on call.
- Offering support to staff, patients, and carers involved, as per NMHS MH post-incident support processes.

9. Monitoring and Evaluation

Compliance with this procedure is mandatory. There are legislative obligations relating to this procedure where a statutory penalty may apply for non-compliance

Procedural compliance will be evaluated during clinical incident investigations or clinical reviews, as required.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed **two (2) years** using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

10. Definitions

Terms	Definitions
Absence without leave (known as “absent without leave” or “AWOL” when referring to an individual consumer)	As defined in s.97 of the <i>MHA 2014</i>
Apprehension and Return Order	As defined in s. 97 of the <i>MHA 2014</i>
Health professional	As defined under Part 2, Division 1 of the <i>MHA 2014</i> .
Mental Health Consumer	A consumer who has had contact with, or been treated by, public mental health services, including mental health services provided in emergency departments.
Missing Person	The Chief Psychiatrist defines a “ <i>missing person</i> ” as “Any voluntary psychiatric patient at high risk of harm who is missing from a mental health service, general hospital or emergency department, without agreement of or authorised by staff.

Refer to [NMHS Policy Document Glossary of Terms](#) for any other frequently used terms.

11. Related Documents

Legislation	• Mental Health Act WA 2014 (MHA 2014) • Criminal Law (Mental Impairment) Act 2023 (CLMI Act) • Prisons Act 1981 (WA)
WA Health mandatory policies	• MP 0012/16 Mental Health Consumers Who Are Missing or Absent Without Leave Policy ○ DoH Procedures for Involving Police When Mental Health Consumers Are Missing or Absent without Leave • MP 0122/19 Clinical Incident Management Policy 2019 • MP 0181/24 Safety Planning for Mental Health Consumers Policy • MP 0155/21 State-wide Standardised Clinical Documentation for Mental Health Services • MP 0063/17 Road-Based Transport for Mental Health Consumers Policy ○ Mental Health Transport Risk Rating Form ○ Form 4A – Transport Order ○ Form 7D – Apprehension and Return Order • DoH MP 0099/18 Community Mental Health Status Assessments: Role of the Mental Health Clinicians Policy
NMHS MHS Policy documents	• MH Clinical Risk Assessment and Management Policy • MHS Inpatient Hospital Campus Access and Community Outings Procedure • MHS Community - Did Not Attend (DNA) Community Appointment Procedure • MH Consumer Declining Assessment or Follow-up Procedure • MHS Community Adult- Morning Acuity Meeting Procedure

12. Document Control

Document Version Control			
Date	Version	Author	Notes
31/05/2018	3.0	A/Policy Officer	Updated template. Revision to text to reflect inclusion of Missing Person Event Flowchart (previously "Absconders Event Flowchart" from NMHSMH AW Clinical Incident Policy), clarification of revision of voluntary patient status.
26/09/2018	3.1	A/Policy and Audit Officer	Inclusion of references to Mental health, NMHS, and DoH Clinical Care of People who may be Suicidal and MP0086/18 Recognising and Responding to Acute Deterioration policy documents.
01/08/2022	3.2	Nurse Coordinator SFMHS	Scheduled review, minor amendments, updated references to overarching DoH Missing Persons Policy, SAC notification flow chart updated, changed from Policy to Procedure, template and links updated.
08/08/2022	3.3	Director of Medical Services MHPHDS	Minor amendment, update of link to Community Mental Health Status Assessments: Role of Mental Health Clinician Policy
17/04/2023	3.4	Director of Nursing MHPHDS	Additional information for staff added to 5.1.1 Interventions/Actions when a patient is missing or suspected of being missing from an inpatient Unit.
11/11/2025	4.0	Adult Inpatient Operational Co Director	Updated to align with DOH Mental Health Consumers Who Are Missing or Absent Without Leave Policy and DoH Procedures for Involving Police When Mental Health Consumers Are Missing or Absent without Leave including language and definitions in reporting to WA Police Force. Inclusion of SFMHS Service information and Criminal Law (Mental Impairment) Act 2023, update of Missing Persons Event Flow Chart.

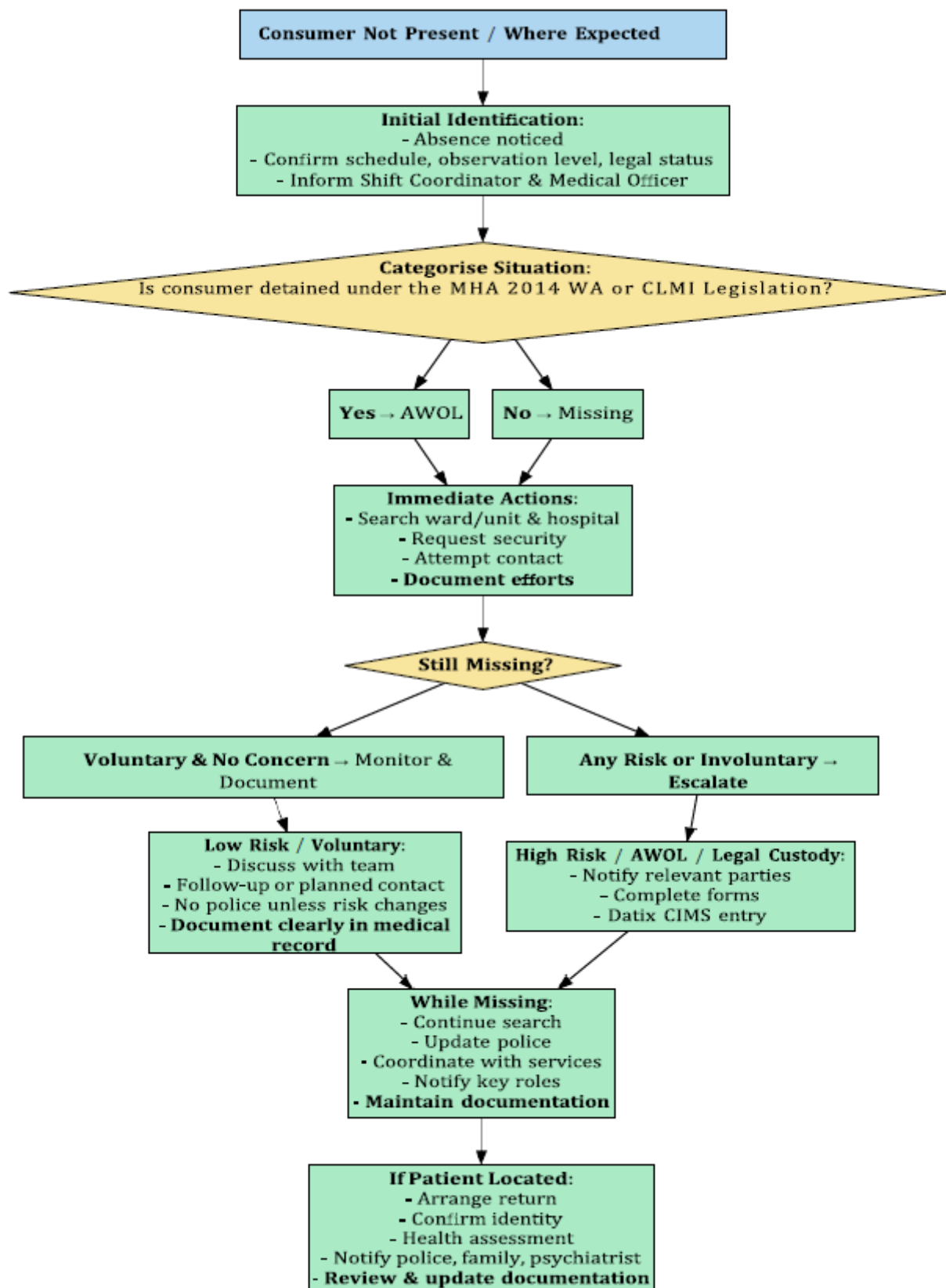
This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Appendix 1: Missing Patients Event Flow Chart



Appendix 2: DATIX CIMS Determination of SAC Rating

Determination of SAC rating:

Patient Status	Patient's Risk Status immediately prior to absence <i>(determined using clinical judgement)</i>	SAC Rating at time of notification of absence	Level of Harm to Patient or Others as a result of absence <i>(actual or near miss)</i>
Involuntary; inpatients, community, order for assessment, referred (AWOL)	High	SAC1	<i>Consider upgrade of SAC rating at the time it becomes known that harm did, or was likely to occur (near miss), in accordance with severity.</i>
	Medium	SAC2	
	Low	SAC3	
Voluntary <i>(Missing Persons)</i>	High	SAC1	<i>Consider upgrade of SAC rating at the time it becomes known that harm did, or was likely to occur (near miss), in accordance with severity.</i>
	Medium	SAC2	
	Low	SAC3	